

# Clinical Decision Support System

## MRI Brain Tumor Analysis Report

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|-------------|----------------------------|
| Patient ID  | ANON_PITUIT                |
| Date/Time   | 2026-05-18T09:34:19.691932 |
| Run ID      | 20260518_093412            |
| Tumor Type  | GLIOMA                     |
| Confidence  | 51.2%                      |
| Radiologist | Approved                   |

### Clinical Summary

Glioma detected. This is a primary brain tumor arising from glial cells. High-grade lesions are fast-growing and require urgent neurosurgical assessment and likely multimodal treatment (surgery, radiation, chemotherapy).

### Recommended Next Steps

1. URGENT neurosurgery consultation
2. MRI with gadolinium contrast + DTI/PWI for surgical planning
3. Neuro-oncology consultation for adjuvant therapy planning
4. Molecular profiling (IDH, MGMT) for prognosis
5. Consider enrollment in clinical trials

### Clinical Guidelines

GLIOMA

Definition: Glioma is a type of tumor that starts in the glial cells of the brain or spine. Glial cells support and protect neurons.

Characteristics:

- Fast-growing, often malignant tumors
- Can originate anywhere in the brain or spinal cord
- Most common primary brain tumors in adults (approximately 30% of all brain tumors)
- Graded I-IV based on aggressiveness (WHO classification); Grade IV (glioblastoma) is the most aggressive
- Often infiltrative, making complete surgical resection difficult

Typical Imaging Findings (MRI):

- Irregular, poorly defined borders
- Heterogeneous signal intensity on T1/T2 sequences
- Ring enhancement with contrast in high-grade gliomas
- Surrounding vasogenic edema (T2/FLAIR hyperintensity)
- Mass effect with midline shift in large tumors
- Central necrosis in glioblastoma

Recommended Next Steps:

1. Confirm diagnosis with stereotactic biopsy if resection is not feasible
2. MRI with and without gadolinium contrast for full staging
3. Advanced MRI sequences: DTI (diffusion tensor imaging) for white matter tracts, PWI (perfusion-weighted imaging) for vascularity
4. URGENT referral to neurosurgery for surgical assessment and possible resection
5. Neuro-oncology consultation for adjuvant chemoradiation planning
6. Consider molecular profiling (IDH mutation, MGMT methylation) for prognosis and treatment
7. Enrollment in clinical trials if available

Prognosis:

- Depends heavily on WHO grade, molecular markers, and patient age
- Grade II: median survival 5-15 years
- Grade III: median survival 2-5 years
- Grade IV (glioblastoma): median survival 14-16 months with standard Stupp protocol (surgery + TMZ + radiation)
- IDH-mutant tumors carry better prognosis than IDH-wildtype

Treatment:

- Maximal safe surgical resection (if feasible)
- Fractionated external beam radiation therapy (60 Gy in 30 fractions for GBM)
- Temozolomide (TMZ) chemotherapy concurrent with and adjuvant to radiation
- Tumor treating fields (TTFields) for recurrence...

*This report is generated by an AI-assisted clinical decision-support system. It is NOT a substitute for professional medical diagnosis. All findings must be confirmed by a qualified radiologist or physician.*